Groin Pain in the Athlete

Background

- Athletic groin pain most commonly occurs in athletes participating in sports that require rapid movement in multiple directions (for example, hockey, soccer, wrestling, etc.).
- The groin is a complex anatomical region. It consists of several joints and muscle attachments and several major nerves, arteries, and veins. This makes injury diagnosis and treatment difficult.
- Diagnosis is made through a review of your history, and an exam that commonly includes an ultrasound evaluation, MRI, and X-Rays.
- The most common injuries that cause groin pain include:
  - » muscle strains (ranging from mild to severe)
  - » impingement (pinching of structures in the joint)
  - » tendinopathy (or tendonitis)
  - » dysfunction or misalignment of your pelvic bones

Latest Research

- Recent research suggests that completing a groin- and core-strengthening program might help prevent groin injuries in some sports (such as soccer and hockey).
- Rehabilitation has been proven to work well for most people. However, some people may experience limited improvement with exercise and physical therapy alone.
- The use of therapeutic injections might be helpful in assisting with persistent athletic groin pain. Examples of injection therapies include platelet-rich plasma (PRP) and prolotherapy.

Practical Tips for Success

- Complete all the exercises and stretches on both sides.
- Be patient with your progress. Groin injuries can take many weeks to months to improve, even with regular rehabilitation.

Rehab Instructions

Adductor stretch

Do not perform if you have an adductor muscle strain. Skip this stretch and move on to the next exercises.

❶ Stand with your toes pointed forward and your feet shoulder-width apart. Lunge to one side and straighten out your other leg. Keep that heel on the floor and point the toes upward. Sit back into the stretch.

❷ To increase the stretch, gently turn your body toward the straightened leg. Keep your back straight and your chest upright.

❸ Hold 30–45 seconds.

☐ Complete 3 sets on each leg and perform 1x every other day.

Adductor Stretch



Standing hip 3-way strengthening: Abduction (out), adduction (in), and extension (back)

❶ Securely attach a resistance band to a piece of fixed, heavy furniture. Use a chair or countertop for balance as needed. Keep your hips level.

Abduction: Stand and place your outside leg inside the loop. Move that leg out (see picture), away from where the band is attached.

Abduction



Adduction: Turn around so the resistance band is now around your inside leg. Move your leg away from where the band is attached.

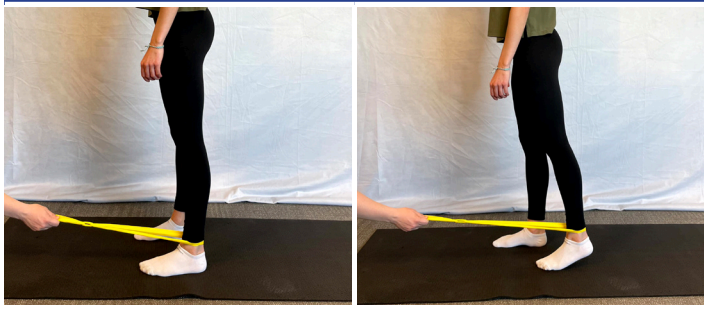
Adduction



Extension: Turn and face where the band is attached and move your leg backwards.

☐ Complete 3 sets of 12 reps in each direction 1x a day on each leg.

Extension



Planks

❶ Lie on the floor face down with your body fully extended and your feet no more than shoulder-width apart.

❷ Raise up on your forearms with your elbows under your shoulders.

Prone plank on hands: Keep your arms extended, with your hands under your shoulders, hips in line with your torso, and legs while on your toes. To advance to prone plank on elbows, rest your forearms on the ground with your elbows under your shoulders.

Prone plank on hand and elbows



Side plank with knees bent: Start with one arm under your side with your elbow directly below your shoulder. Shoulders, hips, and knees should be in a line. Lift yourself up by leading with your torso on the side you are lying on. To advance to side plank with knees straight, straighten your legs and stack one foot on top of the other.

❸ Hold each position for 10–30 seconds.

☐ Complete 3–5 sets 1x a day every other day.

- You may advance to prone plank on elbows and side plank with knees straight if no pain or soreness is felt and when the basic-level exercises are too easy.

Side-lying plank on knees and with knees straight



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